Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 07-Apr-2026

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1. Administrative & Study Identification

Official Study Title: A Phase III, Open-Label, Randomized Study to Evaluate the Efficacy and Safety of Adjuvant Alectinib Versus Adjuvant Platinum-Based Chemotherapy in Patients With Completely Resected Stage IB (Tumors ≥ 4 cm) to Stage IIIA Anaplastic Lymphoma Kinase-Positive Non-Small Cell Lung Cancer **Short Study Title:** A Study Comparing Adjuvant Alectinib Versus Adjuvant Platinum-Based Chemotherapy in Patients With ALK Positive Non-Small Cell Lung Cancer (ALINA Study) **Posting ID:** 14318893389408146 **Protocol Number:** BO40336 **NCT Number:** NCT03456076 **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Sponsor / Company Name:** Genentech / F. Hoffmann-La Roche Ltd (Roche) **Investigational Product:** Alecensa (alectinib) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of adjuvant alectinib compared with platinum-based chemotherapy in improving disease-free survival (DFS) in patients with completely resected ALK-positive non-small cell lung cancer (NSCLC). **Secondary Objectives:** To evaluate overall survival (OS) and the safety profile, including the percentage of patients experiencing adverse events (AEs) and specific high-grade AEs.

2.2 Background & Rationale To address the significant unmet need for targeted treatments in early-stage disease. Today, about half of all people with early-stage lung cancer still experience a cancer recurrence following surgery, despite standard adjuvant chemotherapy. While targeted therapies have transformed outcomes for stage IV ALK-positive disease, there is a critical need to evaluate whether ALK inhibitors like alectinib can prevent disease recurrence and improve the chances of a cure when administered in the adjuvant (post-surgery) setting.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active-controlled (platinum-based chemotherapy) **Blinding:** Open-label **Randomization:** Randomized (1:1 ratio)

3.2 Planned Enrollment & Duration Target N: 257 patients **Number of Sites:** 113 participating sites globally **Estimated Study Start:** August 2018 **Estimated Primary Completion:** November 2026

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Complete resection of histologically confirmed Stage IB (tumor ≥ 4 cm) to Stage IIIA (T2-3 N0, T1-3 N1, T1-3 N2, T4 N0-1) NSCLC as per UICC/AJCC 7th edition, with negative margins, at 4-12 weeks before enrollment. 3. If mediastinoscopy was not performed preoperatively, it is expected that, at a minimum, mediastinal lymph node systematic sampling will have occurred. 4. Documented ALK-positive disease according to an FDA-approved and CE-marked test. 5. Eligible to receive a platinum-based chemotherapy regimen according to the local labels or guidelines. 6. Eastern Cooperative Oncology Group (ECOG) Performance Status of Grade 0 or 1. 7. Adequate hematologic and renal function. 8. For women of childbearing potential: agreement to remain abstinent or use contraceptive methods with a failure rate of < 1% per year during treatment and for at least 90 days after the last dose of alectinib (or per chemotherapy guidelines). For men: agreement to remain abstinent or use contraceptive measures, and refrain from donating sperm for at least 90 days post-treatment. 9. Willingness and ability to comply with scheduled visits, treatment plans, laboratory tests, and other study procedures.

4.2 Exclusion Criteria 1. Pregnant or breastfeeding, or intending to become pregnant during the study or within 90 days after the last dose of alectinib (or per chemotherapy guidelines). 2. Prior adjuvant radiotherapy for NSCLC. 3. Prior exposure to systemic anti-cancer therapy and ALK inhibitors. 4. Stage IIIA N2 patients that, in the investigator's opinion, should receive post-operative radiotherapy treatment. 5. Known sensitivity to any component of study drug (e.g., patients with galactose intolerance, a congenital lactase deficiency, or glucose-galactose malabsorption). 6. Malignancies other than NSCLC within 5 years prior to enrollment (except for curatively treated basal cell carcinoma of the skin, early GI cancer by endoscopic resection, in situ carcinoma of the cervix, ductal carcinoma in situ, papillary thyroid cancer, or any cured cancer considered to have no impact on DFS/OS). 7. Any GI disorder that may affect absorption of oral medications (e.g., malabsorption syndrome or status post-major bowel resection). 8. Liver disease characterized by AST and ALT $\ge 3 \times$ upper limit of normal, impaired excretory/synthetic function, or decompensated liver disease (coagulopathy, hepatic encephalopathy, hypoalbuminemia, ascites, bleeding from esophageal varices, or active viral/autoimmune/alcoholic hepatitis). 9. Japanese patients participating in serial/intensive PK sample collection only: administration of strong/potent CYP450 3A inhibitors or inducers within 14 days prior to the first dose of study treatment and while on treatment up to Week 3. 10. Patients with symptomatic bradycardia. 11. History of organ transplant. 12. Known HIV positivity or AIDS-related illness. 13. Any clinically significant concomitant disease, condition, or psychological/

geographical restriction potentially hampering compliance with the study protocol or posing unacceptable risk.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - *Experimental Arm:* Alectinib 600 mg administered orally twice daily (BID) taken with food. - *Control Arm:* Protocol-specified platinum-based chemotherapy (e.g., cisplatin [Trade name: Platinol, ATC Code: L01XA01]) administered every 21 days. **Route of Administration:** Oral (alectinib) or Intravenous (chemotherapy). **Duration of Treatment:** Up to 24 months for the alectinib arm; 4 cycles for the chemotherapy control arm.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care and therapies for co-morbidities that do not interact with study drugs. **Prohibited:** Prior ALK inhibitors, prior systemic anti-cancer therapies, and strong/potent CYP450 3A inhibitors or inducers (especially for specified PK sub-groups).

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor Mutational Status (ALK-testing), Baseline Scans | | **Baseline** | Day 0 | Randomization, First Dose of Study Drug | | **Interim** | Every 21 Days (Control) / Regular Intervals (Exp) | Safety Labs, Efficacy Assessment, Dosing & Tolerability Checks | | **End of Study** | Up to 5 Years Follow-up | End of treatment visit, survival follow-up calls/visits every 6 months until disease recurrence or death |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Disease-free survival (DFS), as Assessed by the Investigator, defined as the time from randomization to the first documented recurrence of disease or new primary NSCLC as determined by the investigator through use of an integrated assessment of radiographic data, biopsy sample results (if clinically feasible), and clinical status or death from any cause, whichever occurs first. **Measurement Method:** Radiologic imaging and investigator assessment.

7.2 Secondary Endpoints 1. Overall Survival (OS). 2. Percentage of Participants With Adverse Events (AEs). 3. AEs Grade 3-5 With a Difference in Incidence Rate of at Least 2% Between Treatment Arms.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via regular clinic visits, physical examinations, and laboratory panels to track common toxicities (e.g., GI disorders, elevated creatine phosphokinase, transaminase elevations). **Serious Adverse Events (SAEs):** Reported within 24 hours per standard oncology clinical trial regulations. **Data Monitoring Committee (DMC):** Supervised and managed by the sponsor (Genentech / F. Hoffmann-La Roche Ltd).

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy objectives. Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Approximately 257 patients appropriately powered to detect a statistically significant hazard ratio improvement in DFS. **Handling of Missing Data:** Right-censoring applied for patients without a documented DFS or OS event by the end of the follow-up period.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard onsite and remote monitoring throughout the treatment and follow-up phases. **Audit Trail:** Robust electronic case report form (eCRF) locking and tracking for both safety parameters and critical survival data.

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11. Study Identifiers & Governance

Primary ID: BO40336 **Posting ID:** 14318893389408146 **Registry Number:** NCT03456076 **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Sponsor/Lead Organization:** Genentech / F. Hoffmann-La Roche Ltd (Roche) **Study Title:** A Study Comparing Adjuvant Alectinib Versus Adjuvant Platinum-Based Chemotherapy in Patients With ALK Positive Non-Small Cell Lung Cancer **Official Regulatory Title:** A Phase III, Open-Label, Randomized Study to Evaluate the Efficacy and Safety of Adjuvant Alectinib Versus Adjuvant Platinum-Based Chemotherapy in Patients With Completely Resected Stage IB (Tumors ≥ 4 cm) to Stage IIIA Anaplastic Lymphoma Kinase-Positive Non-Small Cell Lung Cancer

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12. Clinical Framework (NSCLC Specific)

Primary Condition: Carcinoma, Non-Small-Cell Lung (Preferred UMLS: Carcinoma, Non-Small-Cell Lung, ALK Positive) **Diagnosis Threshold:** Stage IB (tumor ≥ 4 cm) to IIIA completely resected disease **Medical Methodology:** Adjuvant Post-Surgical Treatment **Optimization Target:** Prevention of disease recurrence and extension of disease-free survival

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13. Study Procedures & Methodology

Management Pathway: Targeted oral therapy (alectinib) vs. standard IV platinum-based chemotherapy (e.g., cisplatin) **Education Protocol (HCP):** Site initiation, investigator meetings, and AE management training **Education Protocol (Patient):** Informed consent, potential side-effect counseling, and treatment compliance education **Quality Audit Mechanism:** Periodic clinical monitoring and independent centralized data review

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14. Observation & Follow-Up Schedule

Total Duration: Up to 5 years follow-up **Onsite Visit Cadence:** Every 21 days for chemotherapy; protocol-specified regular intervals for oral therapy **Remote Monitoring Frequency:** Every 6 months via follow-up telephone calls during the survival phase **Checklist Review Interval:** Routine symptom and AE check at every clinic visit

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY] | | Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |